



Standard of Care

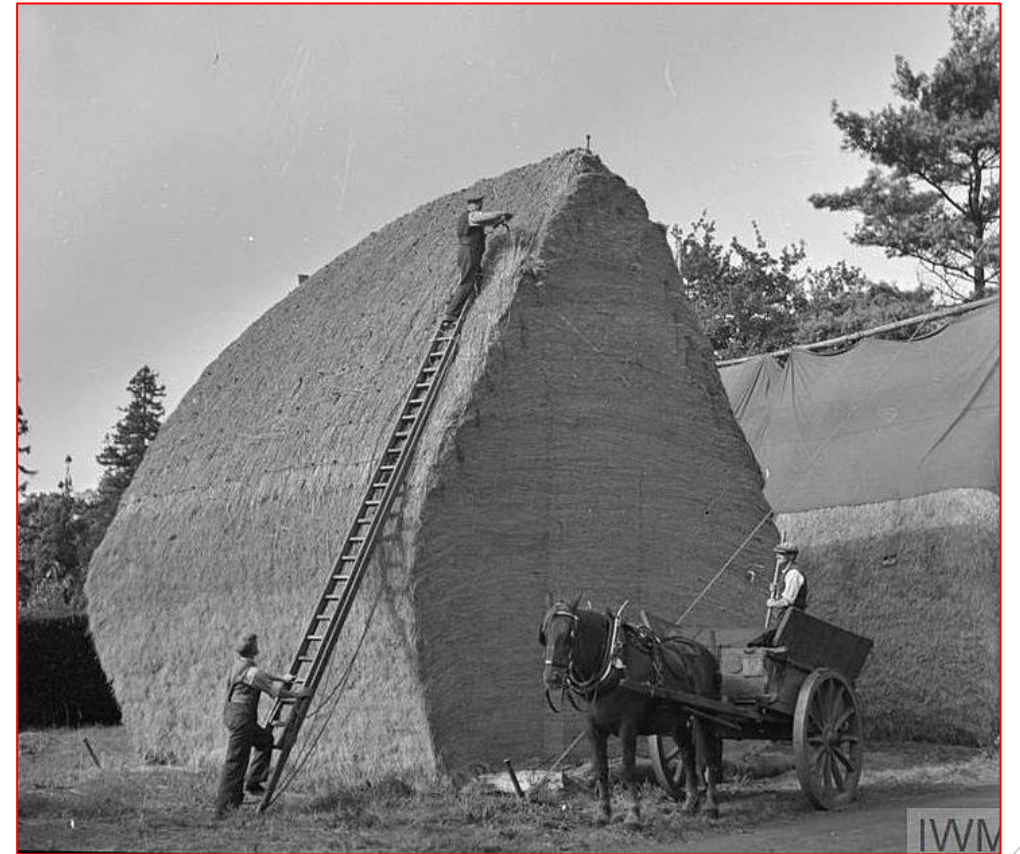
Professor Suzanne Chiodo
Tuesday, September 29, 2026

Introduction

- The duty of care asks, “should the defendant have taken care?”
- The standard of care (SOC) asks, “did the defendant take care?”
- Two questions:
 - What was the standard?
 - Did the defendant meet it?
- The standard is usually that of a ‘reasonable person’

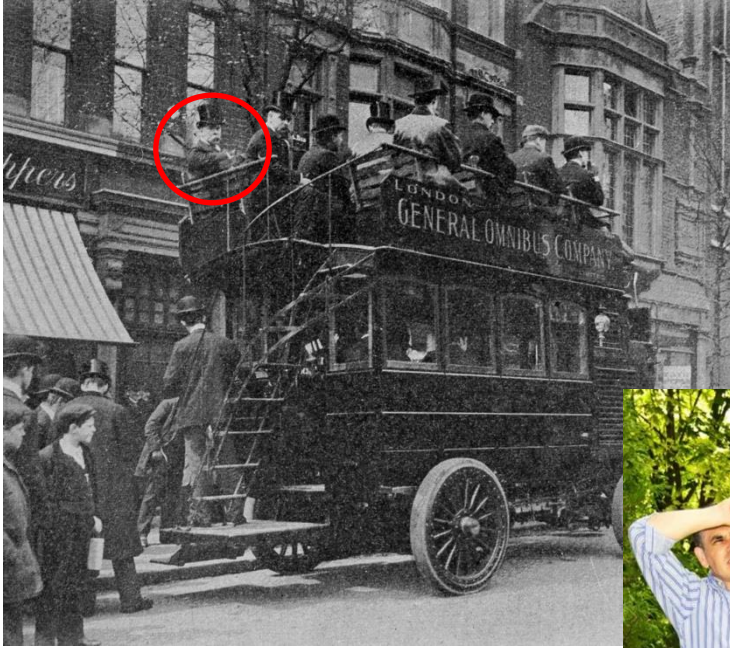
Vaughan v Menlove (1837)

- Articulated 'reasonable person' standard
- What happened?
- What standard did the defendant say he should be held to?
- What did the court decide?
- The defendant must, "adhere to the rule which requires in all cases a regard to caution such as a man [person] of ordinary prudence would observe."



The SOC of the 'Reasonable Person'

- Objective
- Focuses on defendant's conduct
- Defendant's subjective awareness of risk is irrelevant
- Excludes individual characteristics
- A mixture of what ordinary people do, and what judges (or other finders of fact) think they ought to do
- Not a standard of perfection
- A legal construct, based on the 'ordinary' person



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Exceptions to the 'Reasonable Person' Standard

1. **Children**
2. **People with conditions affecting mental functioning**
3. **Persons representing themselves as having superior skill and knowledge**

Children — *McHale v Watson* (1966)

- What happened?
- What did the plaintiff argue?
- What did the Court decide?
- What is the standard of care for children?
- What did the court say about (i) foresight of harm, and (ii) prudence?
- Is there a gendered component?



Children — *McErlean v Sarel* (1987)

- What happened?
- What did the Court decide?
- What standard of care will children be held to when they are engaging in activities that adults engage in?



Mental functioning

- *Buckley v Smith* (1946)
 - What happened?
 - What was the defence?
 - What did the Court decide?
- *Mansfield v Weetabix* (1998)
 - What happened?
 - What did the Court decide?
 - What was the standard of care?



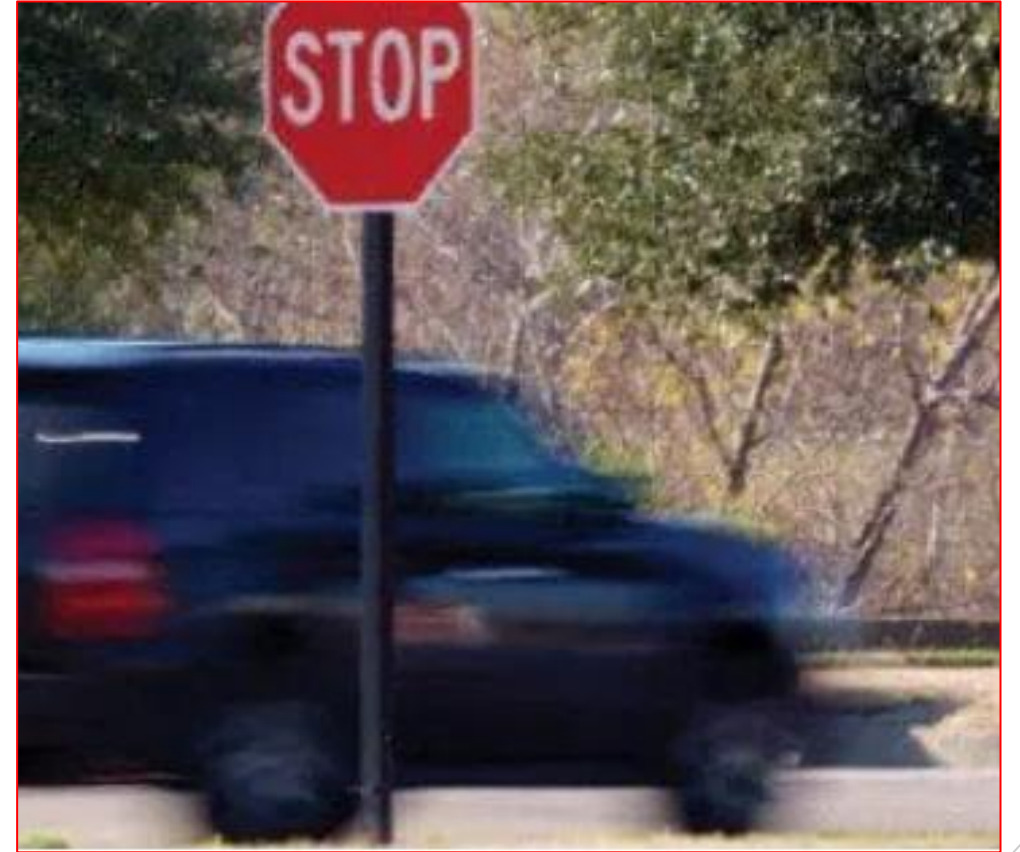
Mental functioning – *Dunnage v Randall* (2016)

- What happened?
- What did the lower court decide?
- What did the Court of Appeal decide?
- What did it say about the standard of care in this case?



Mental functioning – *Moushi v Stephen* (2019)

- Canadian courts have been more generous than English courts
- What happened?
- What did the court decide and why?



Conditions affecting mental functioning

- In order to come under this exception, the defendant must demonstrate that:
 - As a result of their mental illness, the defendant had no capacity to understand or appreciate the duty of care owed at the relevant time; OR
 - As a result of mental illness, the defendant was unable to discharge their duty of care as they had no meaningful control over their actions at the time the relevant conduct fell below the objective standard of care.

Superior Skill and Knowledge

- Those who represent themselves as having special skill & knowledge are held to higher standard that matches that representation
- Standard is that of a reasonably competent and prudent person who belongs to that profession/trade/vocation
- Expert evidence will often be used to establish the standard of care



- What are the advantages of the 'reasonable person' standard?
- What are the disadvantages?
- Should we keep the reasonable person standard, or move to a purely subjective one?



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Factors in Applying Standard of Care

The court will consider several factors when determining whether defendant fell below standard of care:

1. Reasonably foreseeable risk
2. Likelihood of damage occurring
3. Seriousness of threatened harm
4. Cost of preventative measures
5. Utility of defendant's conduct
6. Emergency circumstances
7. Compliance with approved practice/custom

1. Reasonably Foreseeable Risk

- Would a reasonable person have anticipated and guarded against the risk?
- Was the danger obvious to a reasonable person?
- This goes to the level of care the defendant ought to have taken against that particular risk
- However, this is subject to hindsight bias
- Foreseeable risk alone does not indicate that defendant has fallen below the standard of care – the likelihood of damage is also relevant

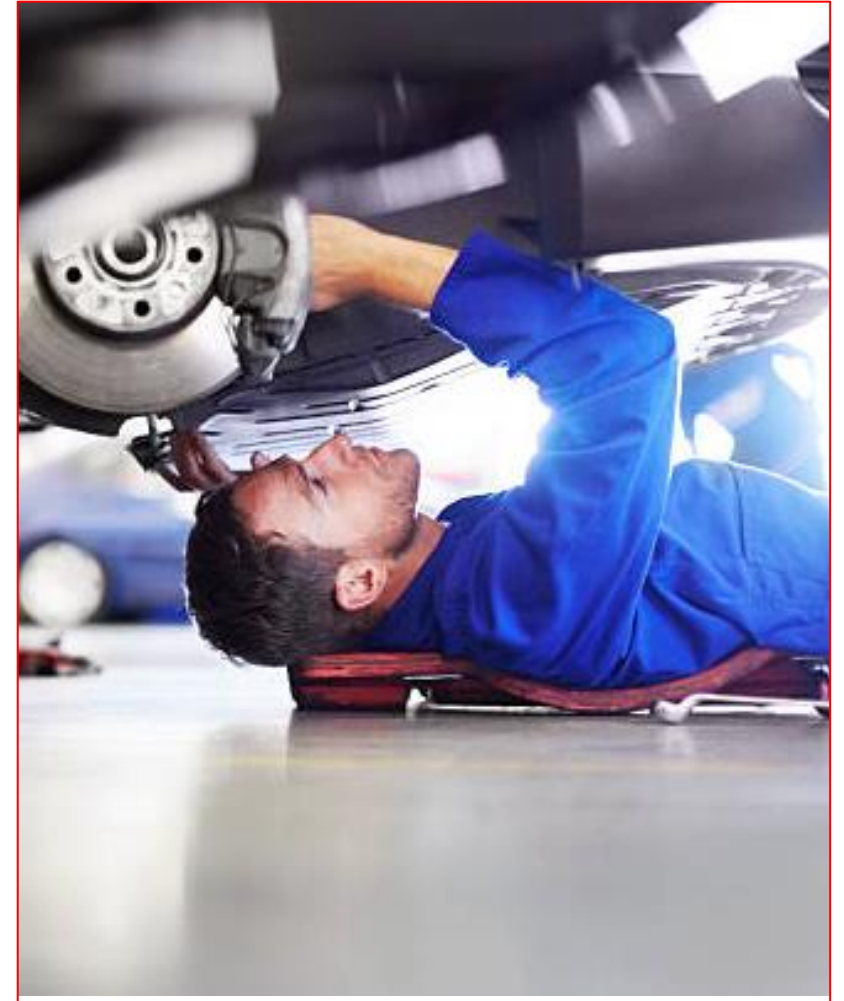
2. Likelihood of damage – *Bolton v Stone*

- What happened?
- What did the plaintiff argue?
- What did the House of Lords say?
- How did it distinguish between different types of foreseeable risk?



3. Seriousness of harm – *Paris v Stepney*

- What happened?
- What did the defendant argue?
- What did the House of Lords say?

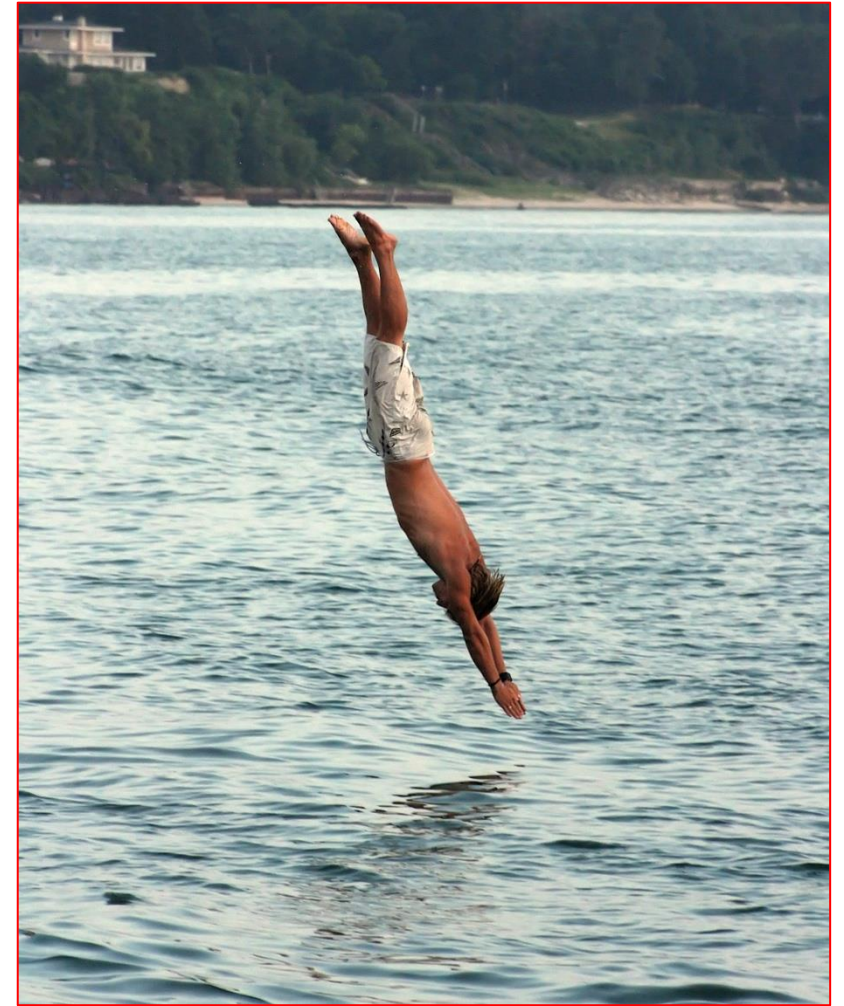


4. Cost of Preventative Measures

- Hand Formula – *United States v Carroll Towing* (1947)
- Defendant falls below standard of care if:
 - The burden of adequate precautions (B) is less than
 - The magnitude of the risk (probability of damage (P) x gravity of resulting injury (L))
 - Formula is $B < (P \times L)$
- Considers the most economically efficient level of accident prevention
- What are its advantages and disadvantages?
- *Latimer v AEC Ltd* (HL 1953) – Weinrib pp 82-83
- *Bolton v Stone* (HL 1951) – Weinrib pp 79-81

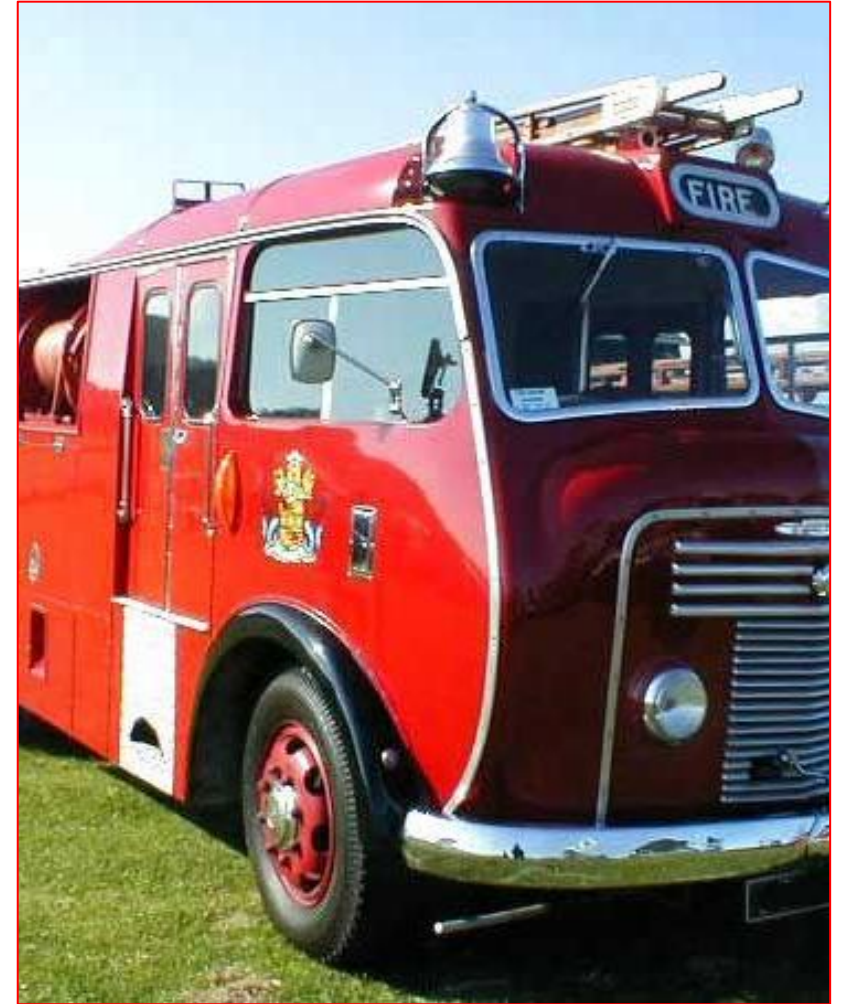
5. Utility of Defendant's Conduct

- *Tomlinson v Congleton BC* (HL 2004)
- What happened?
- In the Court's view, what was the only way to remove the risk? Do you agree?
- What did the court say about the social value of the activity at stake?



6. Emergency Circumstances

- Courts are more lenient on people acting in emergency circumstances
- *Watt v Hertfordshire CC* (1954, England)
- Plaintiff firefighter was injured by a jack that was improperly secured – no time to get the appropriate truck
- Denning LJ held that the attempt to save a life outweighed the risk incurred in taking out the wrong vehicle



7. Compliance with Practice or Custom

- Compliance with standard practice may establish due care
- Finder of fact will also have to assess reasonableness of the custom
- *Saskatchewan Wheat Pool* (SCC 1983) – statutory formulation of the duty can help determine standard of reasonable conduct
- *Trimarco v Klein* (NYCA 1982) – P showed that use of shatterproof glass in bathtubs was common practice, so that his bathtub did not conform to safety standards



7. Compliance with Practice or Custom

- *ter Neuzen v Korn*, [1995] 3 SCR 674
- What happened?
- What was the standard industry practice? Did the defendant comply with it?
- What did the SCC say about whether the standard medical practice was negligent?
- How did it differentiate between complex, uncertain, and highly technical practices, and practices that are easier to understand?
- Are you persuaded by this distinction?

7. Compliance with Practice or Custom

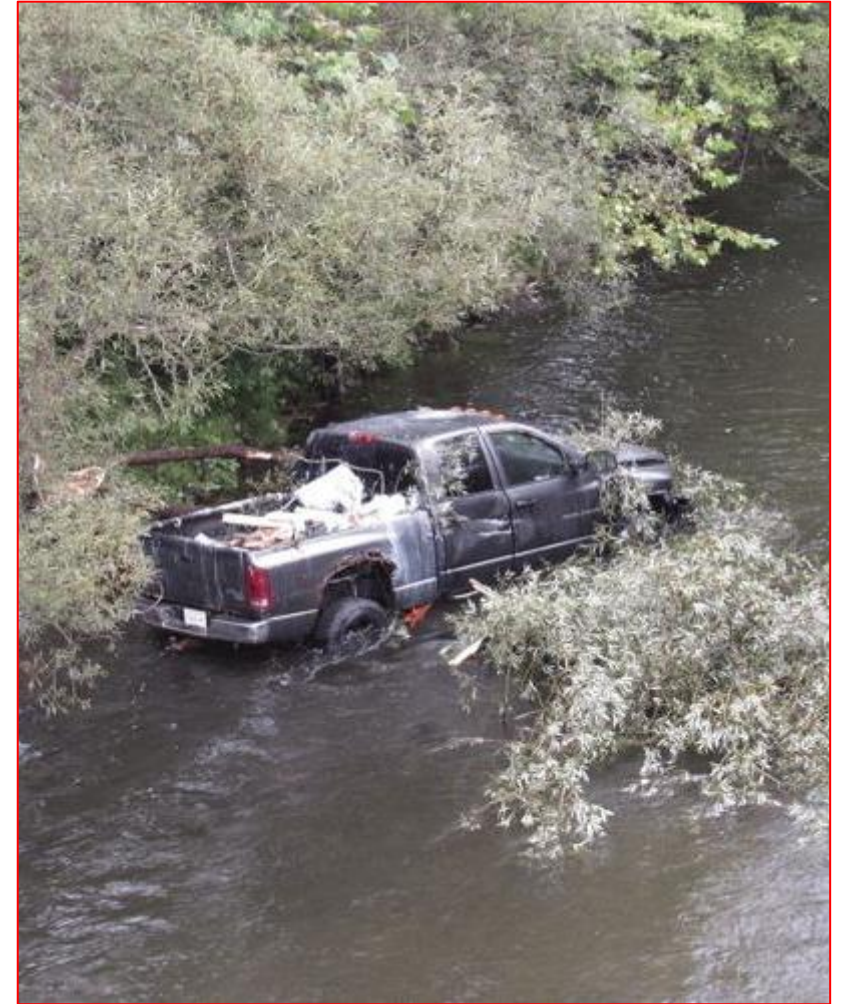
- *Blackwater v Plint*, 2005 SCC 58
- What happened?
- What was question for the SCC regarding the standard of care?
- What did the trial judge conclude, and why?
- What did the plaintiff argue, at trial and on appeal?
- What did the SCC decide?

Res Ipsa Loquitur

- Civil juries used to be much more common in Canada than they are now
- If plaintiff had no direct evidence that defendant fell below standard of care, then the defendant could move for a nonsuit (i.e., so little evidence, there is no case for defendant to answer)
- Doctrine of *res ipsa loquitur* overcame that difficulty – if plaintiff could show circumstantial evidence that SOC was not met, that would be *prima facie* evidence of negligence – then jury could decide or D could rebut
- *Res ipsa loquitur* = ‘the thing speaks for itself’
- Inference of negligence where the event does not ordinarily happen without negligence, and event was under defendant’s sole control

Fontaine v BC, [1998] 1 SCR 424

- SCC reconsidered *res ipsa loquitur*
- What happened?
- Why did the SCC say that use of *res ipsa loquitur* was no longer appropriate?
- What did the SCC say should replace it?



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Next class:
Small Groups (Fact Pattern)